

STEP 1

OPTIMISE STATIN FIRST

Up to Atorvastatin 80mg OR Rosuvastatin 40mg* (or maximum tolerated dose)

- Optimise lifestyle measures
- Ensure adherence to lipid-lowering medications
- Use [statin intolerance pathway](#) if unable to tolerate any dose of statin

STEP 2 Lipid therapy in addition to maximally tolerated statin based on LDL-C levels		
LDL-C Level (mmol/L)	Treatment Action	
Persistently > 4.0 (single cv event)	Referral to lipid clinic for PCSK9 inhibitors	
Persistently > 3.5 (multiple cv events)		
Persistently ≥ 2.6 - 3.5/4.0	Inclisiran	
2.0 - 2.5	Ezetimibe 10mg Once Daily	Consider ezetimibe if LDL-C remains >1.4 after injectables
1.4 - 1.9	Consider Ezetimibe 10mg Once Daily	

Criteria for referral to lipid clinic

First exclude secondary causes: hypothyroidism, uncontrolled diabetes, excess alcohol and nephrotic syndrome

Suspected Familial Hypercholesteremia	Triglycerides (TG)
Any of • Total cholesterol ≥ 7.5 mmol/L • LDL-C ≥ 4.9 mmol/L	• ≥ 20 mmol/L once OR • ≥ 10 mmol/L twice (in absence of diabetes/alcohol)

[Icosapent Ethyl](#) can also be prescribed if,

- Fasting TG 1.7-5.6 mmol/L &
- LDL-C 1.04-2.60 mmol/L &
- Taking Statin therapy

Green (specialist advice)

THE LOWER THE LDL-C, THE LOWER THE CV RISK - THERE IS NO LOWER LIMIT

Initially review historical statin prescriptions and consider switching to recommended statins at the appropriate dose, unless there is a documented history of intolerance or adverse effects.

*Use Rosuvastatin 20mg OD if on [clopidogrel or of Asian origin](#)

- [Bempedoic Acid](#) is not recommended alongside statin therapy
- Persistently raised = two levels above threshold taken at least 3 months apart
- Repeat cholesterol and LFTs 6-8 weeks after initiation or change of statins
- Use non-fasting samples for routine monitoring
- If calculated LDL is unobtainable (triglycerides >4.5), repeat fasting